

# ABSENT FROM SCHOOL. PRESENT IN A SYSTEM.

## Absconding as a developmental and ecological signal

A bioecological case formulation and dual-focus family intervention for the Aisyah family

### DISTINCTION DEFENCE

Bronfenbrenner PPCT, functional casework and family intervention

#### Central proposition

D2's absconding may function as an immediate escape from relational strain. Sustainable change therefore requires safer proximal processes at home and coordinated support across school, kin and social services.

# Case at a glance

Observed facts, provisional meanings and missing information

## Observed presentation

D2, age 17: repeated absence from school, declining grades and absconding to her maternal aunt.  
D3, age 15, has begun accompanying her.

## Family context

Eight residents in a two-room rental flat. One adult works rotating shifts. Financial assistance has ended. D1 carries adult-like responsibilities.

## Working hypothesis

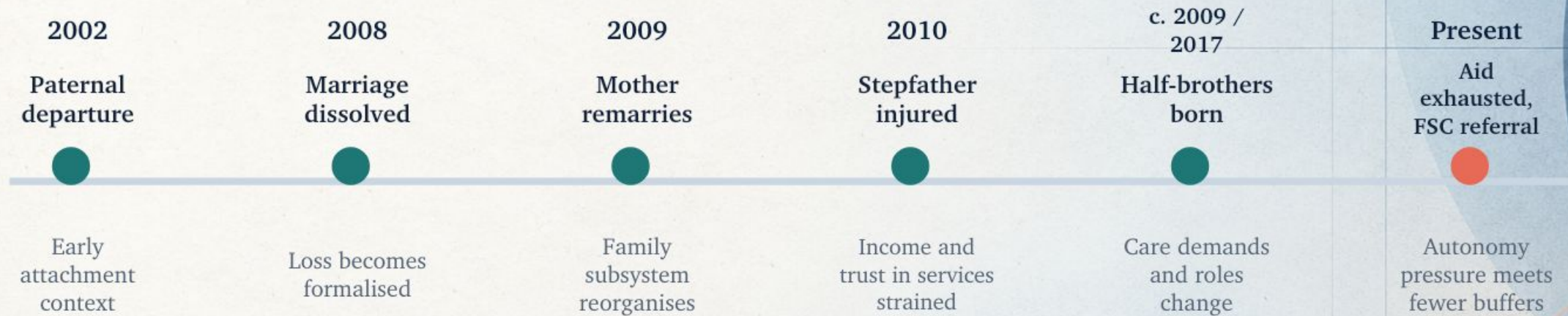
Absconding may reduce conflict quickly and provide perceived safety, autonomy or recognition. The pattern can persist when return arrangements remain unchanged.

## Assessment questions that must remain open

-  Where does D2 stay, and is each location safe?
-  What happens immediately before leaving and after returning?
-  How does D2 describe fairness, belonging and control?
-  Which school, health, peer or online factors remain unknown?

# Family transitions across D2's development

Timing matters because repeated transitions can change the quality and availability of daily relationships



## CHRONOSYSTEM READING

The chronology suggests cumulative change rather than a single trigger. Verify date inconsistencies, the quality of attachment relationships and D2's own account before linking current behaviour to earlier transitions.

# Relational architecture

Three relationship patterns organise stress and support

## Executive overload

Mother and D1 carry much of the household management. D1's parent-like role may protect the family while constraining her own development.

## Conflict and escape

Mother-D2 conflict may form a coercive loop: escalation, departure, short-term relief, then unresolved return.

## Kin refuge

The maternal aunt appears to offer attachment and practical shelter. Her role can become a bridge for repair if safety and authority are negotiated.



# The household is connected, but the connections do not yet coordinate

The intervention target includes the links between settings, not only behaviour within the home



# UNPACKING THE FUNCTIONAL THEORY IN SOCIAL WORK

## 1. THE WILL AND HUMAN AGENCY



Belief in a person's innate, creative power to determine their own life path.

## 2. CLIENT'S RIGHT TO SELF-DETERMINATION



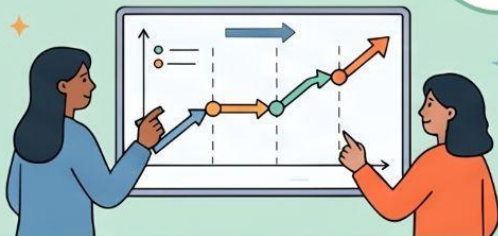
The client is the primary author of their change and controls the therapeutic outcome.

## 3. START WHERE THE CLIENT IS



The focus is on the client's current, conscious needs, not a hidden diagnosis.

## 4. ONGOING, EVOLVING ASSESSMENT



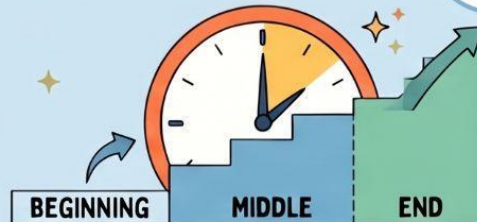
Assessment is a dynamic process that grows within the helping relationship.

## 5. IMPORTANCE OF RELATIONSHIP



Help emerges through the experiencing and transactions within the unique helping bond.

## 6. TIME AS AN ORGANISING COMPONENT



Intervention is structured with three phases to drive growth through planned separation.

# KEY TECHNIQUES OF FUNCTIONAL THEORY IN SOCIAL WORK

## 1. FOCUS ON THE HERE AND NOW

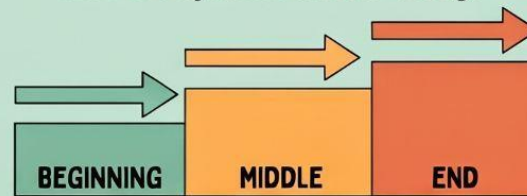
Work centers on the active relationship, not the past



Interventions focus on present interactions and immediate feelings within the helping dynamic.

## 2. STRUCTURED TIME PHASES

Time is a dynamic tool for change



Uses phases to build momentum and drive client growth through planned separation (termination).

## 3. EXPERIENTIAL RELATIONSHIP-BUILDING

Help comes from lived interaction



The worker supports autonomy by "experiencing" the client, not through intellectual analysis.

## 4. DUAL BOUNDARY SETTING

Framing agency and social limits

**AGENCY LIMITS**  
policy,  
procedures



**SOCIAL LIMITS**  
legal rules,  
anti-harm

Clarifies expectations using agency function and legal "Boundaries of Stability" to encourage choice.

## 5. RELATIONAL ENGAGEMENT & TRUST

Authentic connection for trust-building



Forms a secure, authentic bond allowing clients to safely explore choices and overcome tension.

## SYNTHESIS OF TECHNIQUES

How they work together



Integration of these methods mobilizes client Will, respect autonomy, and facilitates growth within safe parameters.

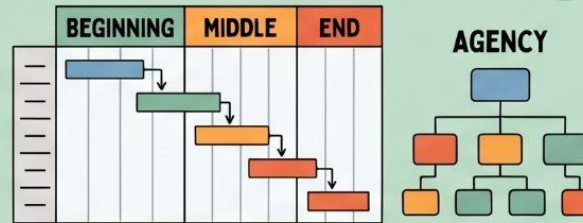
# CRITICAL EVALUATION: STRENGTHS

## 1. EMPOWERED CLIENT AUTONOMY



Recognizes clients as active decision-makers with the internal will to change.

## 2. CLEAR STRUCTURE & FOCUS



Uses defined time limits and specific agency functions to maintain organization and productivity.

## 3. PRACTICAL RESOURCE-DRIVEN



Connects clients directly to concrete, available community resources and programs.

## 4. POSITIVE GROWTH-ORIENTED



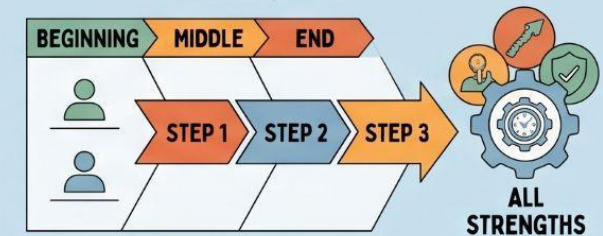
Emphasizes current strengths, problem-solving, and future potential.

## SYNTHESIS OF STRENGTHS: Empowering Client Change



Integration of strengths mobilizes client decision-making, leverages structure, and connects to resources for tangible progress.

## INTEGRATED CASE APPLICATION: The Complete Process



Integration of strengths mobilizes client decision-making, leverages structure, and connects to resources for tangible progress.

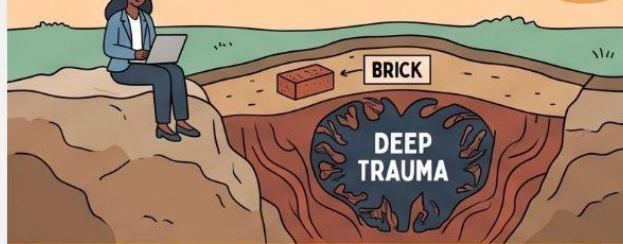
# CRITICAL EVALUATION: LIMITATIONS

## 1. RIGID AGENCY FOCUS



Can restrict the social worker's helpfulness if the client's needs fall outside the strict, pre-defined functions of the agency

## 2. IGNORES DEEP TRAUMA



Focuses heavily on the present, making it less effective for clients requiring deep, long-term psychological healing from past experiences

## 3. OVEREMPHASIZES WILLPOWER



Assumes all clients have equal capacity to exercise free will, which may overlook severe mental health crises or systemic oppression

## 4. STRICT TIME CONSTRAINTS



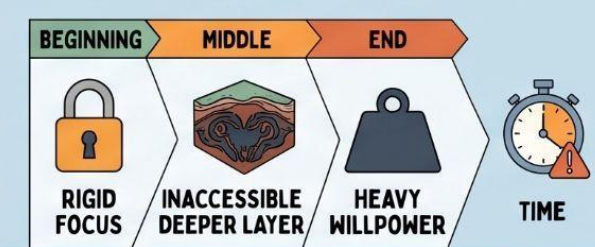
The artificial enforcement of rigid time phases may not align with the messy, unpredictable nature of human crisis

## SYNTHESIS OF LIMITATIONS: Navigating Structural & Personal Limits



Integration of limitations highlights rigid frameworks, personal biases, structural barriers, and time pressures.

## INTEGRATED CASE APPLICATION: Limits in Practice



Application shows challenges in multi-step progress when agency limits are hit, trauma is ignored, willpower is insufficient, and time runs out.

# Functional theory: interpreting behaviour in context

The formulation stays provisional and tests meaning before assigning motivation

| Functional lens        | Case interpretation                                                                                                  | Practice implication                                                                                           |
|------------------------|----------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------|
| Will and avoidance     | Missed appointments and documents may conserve limited energy and preserve a sense of control under strain.          | Test this meaning collaboratively. Agree on smaller commitments before describing the behaviour as resistance. |
| Family role allocation | D1 carries parent-like responsibilities while the father has a limited role in household governance.                 | Clarify adult responsibilities and restore age-appropriate roles without assigning blame.                      |
| Agency structure       | Forms, scheduled visits and eligibility criteria create predictable boundaries around assistance.                    | Explain why each requirement matters, pace the paperwork and use consistency as part of the intervention.      |
| Boundary crisis        | D2’s stays with her aunt may provide distance when physical and emotional boundaries at home feel unsafe or unclear. | Assess safety and negotiate a planned pause, contact and return protocol with D2 and the caregivers.           |

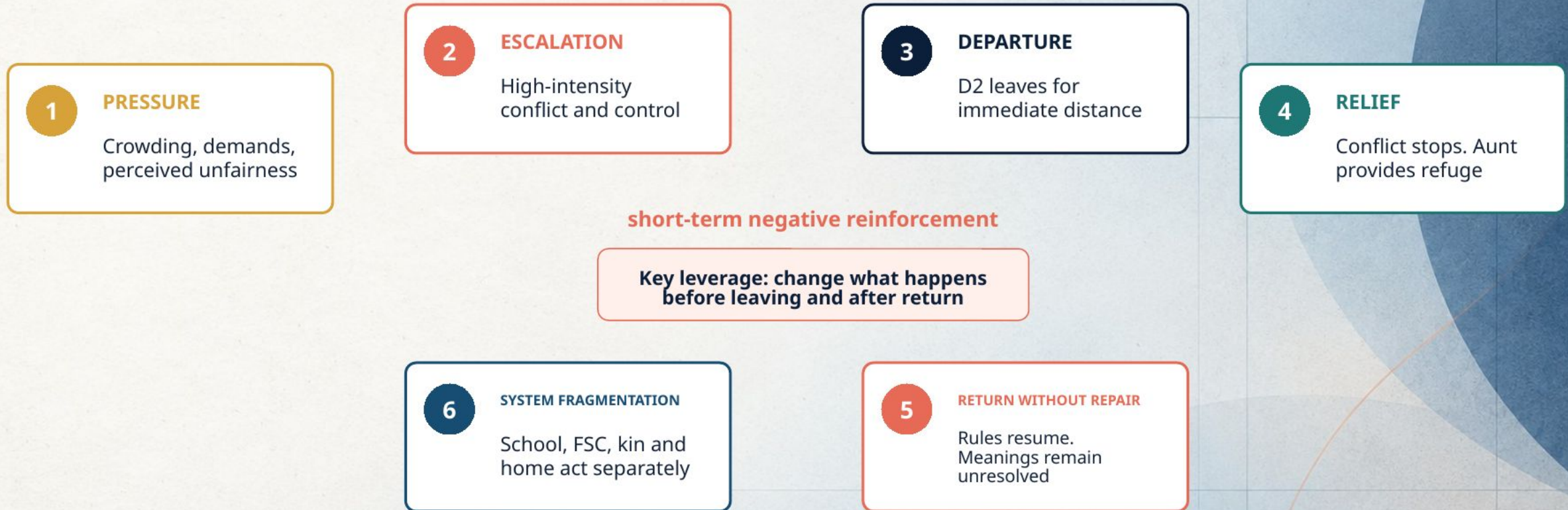
# Restoring family function through structured casework

Each action links a concrete task to an observable review marker

| Intervention priority             | Concrete casework action                                                                                          | Intended change and review marker                                                               |
|-----------------------------------|-------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------|
| Contracted engagement             | Co-write a working agreement covering fortnightly sessions, one attainable task and contact between appointments. | Attendance and task completion improve. Reasons for missed commitments guide adaptation.        |
| Manageable problem-solving        | Sequence urgent practical work, beginning with one agreed application or document set.                            | Incomplete tasks decrease and Aisyah reports less overwhelm during reviews.                     |
| Rebalanced family roles           | Connect D1 with education or employment support. Agree which caregiving responsibilities remain with the adults.  | D1 carries fewer adult-like tasks while caregivers complete the responsibilities they accepted. |
| Planned time frame and transition | Use a 16-session frame with scheduled reviews, a transition plan and a route back if risk increases.              | Progress continues with fewer crisis-led contacts and clearer use of community supports.        |

# Why the absconding pattern may persist

A provisional maintenance cycle integrates meaning, interaction and ecology



# PPCT as the primary explanatory framework

Development emerges through recurring interaction between the person and changing environments over time

## PROCESS

Conflict, caregiving,  
school contact and  
repair routines

## PERSON

D2's agency,  
developmental timing,  
beliefs and resources

## CONTEXT

Home, aunt, school,  
FSC, employment  
and policy

## TIME

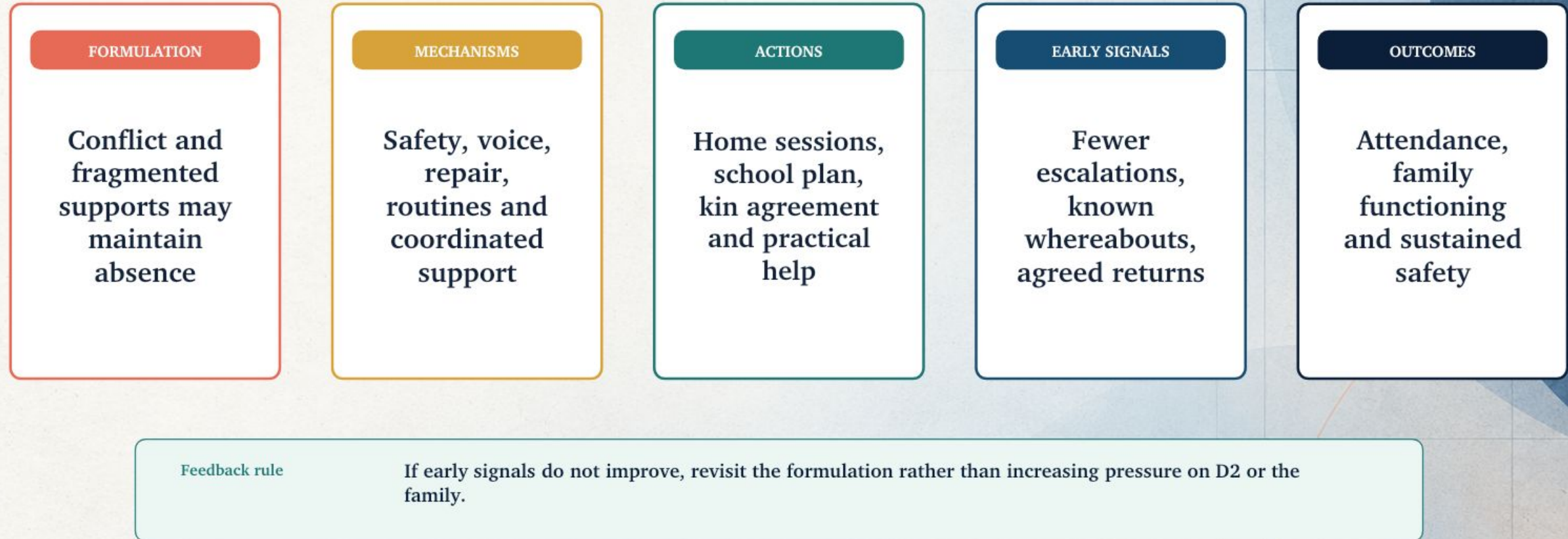
Episode sequence,  
transitions and  
intervention phases

### Case implication

**The unit of change is the recurring interaction pattern. Person-only counselling may help D2 cope, but outcomes also depend on family repair, school re-entry and service coordination.**

# From formulation to change logic

The intervention changes recurring interactions, then tests whether functioning improves



# BERSAMA: a 16-week dual-focus intervention

Building Engagement, Relational Safety, Autonomy, Mastery and Attendance

WEEKS 0–2

## ENGAGE

Safety, consent,  
separate voices

WEEKS 3–4

## REFRAME

Map sequences and  
reduce blame

WEEKS 5–12

## CHANGE

Practise repair,  
boundaries and  
routines

WEEKS 13–16

## GENERALISE

School re-entry and  
relapse plan

### DUAL FOCUS

#### Person-directed

D2's safety, voice, coping and education  
goals

#### Environment-directed

family patterns, kin agreements, school  
and material supports

# BERSAMA phase plan, responsibilities and indicators

A 16-session pathway turns formulation into observable family change

| Time Phase | Action Step                  | Functional Rationale                                                                                 |
|------------|------------------------------|------------------------------------------------------------------------------------------------------|
| Beginning  | Contracted engagement        | Co-write a clear agreement on session rhythm, immediate goals and paperwork responsibilities.        |
| Beginning  | Planned time frame           | Set a fixed 16-session frame with review points and an agreed route back if risk increases.          |
| Middle     | Manageable tasks             | Sequence one urgent practical task at a time to reduce overwhelm and build completion.               |
| Middle     | Rebalanced roles             | Support D1’s education or employment pathway while adults resume agreed caregiving responsibilities. |
| End        | Consolidation and transition | Review indicators, rehearse responses to setbacks, confirm community supports and close responsibly. |

# Measurement tests the formulation, not the family's compliance

Combine behavioural, relational and person-defined indicators

| Outcome domain      | Baseline                                   | Weekly or fortnightly indicator                            | Review point   |
|---------------------|--------------------------------------------|------------------------------------------------------------|----------------|
| Safety              | Unknown locations and return pattern       | Nights at known safe location and contact plan used        | Weekly         |
| Education           | Attendance and grades declining            | Attendance percentage, punctuality and D2-rated school fit | Weeks 4, 8, 16 |
| Family interaction  | Escalation and leaving                     | Conflict frequency, intensity and repair completion        | Fortnightly    |
| D2-defined progress | Feeling misunderstood and unfairly treated | 0–10 voice, fairness and hope ratings plus narrative       | Every session  |
| System coordination | Fragmented agency response                 | Named lead, shared plan and completed warm referrals       | Weeks 2, 8, 16 |

Safety

Attendance

Family repair

D2-defined goal

EVALUATION

Illustrative dashboard structure only. Targets must be co-defined after baseline assessment

# Session architecture protects voice and builds family capacity

A single family meeting is insufficient for a conflict pattern shaped by power and safety

## D2 space

Separate check-in for goals, confidentiality limits, safety and consent. Translate concerns into language she accepts.

## Caregiver space

Support stress regulation, clarify authority and practise boundaries without humiliation or threats.

## Joint practice

Short, structured conversations with a pause plan. Rehearse repair and return before testing harder topics.

## A 60-minute family session

10 min

Check safety

10 min

Name progress

20 min

Practise one sequence

10 min

Agree the week

10 min

Private close

# A defensible social work proposition

The most useful explanation is the one that creates safer, testable opportunities for change

**D2's absence can be read as communication within a strained ecology. The response therefore combines immediate safety, a credible role for D2, family interaction change and coordinated school-social support.**

Distinction claim

Theory is translated into assessment questions, actions, indicators and boundaries.

## Selected references

- Bronfenbrenner, U., & Morris, P. A. (2006). The bioecological model of human development.
- Conger, R. D., et al. (1992). A family process model of economic hardship and adjustment of early adolescent boys. *Child Development*, 63, 526–541.
- Gubbels, J., van der Put, C. E., & Assink, M. (2019). Risk factors for school absenteeism and dropout. *Journal of Youth and Adolescence*, 48, 1637–1667.
- Littell, J. H., et al. (2023). Functional Family Therapy for families of youth with behaviour problems. *Campbell Systematic Reviews*.
- Ministry of Education Singapore. Counselling and Student Welfare, attendance support and parliamentary replies.
- Ministry of Social and Family Development Singapore. ComCare and service directories.
- Singapore Statutes Online. Children and Young Persons Act 1993.

# Thank you. Q & A

*Note: Additional slides should be read in conjunction with the main slide deck. They provide context for biopsychosocial ecological research and analysis, and for the formulation of the integrated phased plan.*

# Integrated biopsychosocial-ecological formulation

Each domain links a case marker to an assessment need and a modifiable process

| Domain                     | Case marker                                   | Assessment focus                                               | Changeable process                         |
|----------------------------|-----------------------------------------------|----------------------------------------------------------------|--------------------------------------------|
| Biological / developmental | Age 17. Sleep and health unknown              | Development, sleep, health, substance exposure, neurodiversity | Routines, health access, regulation skills |
| Psychological              | Feels misunderstood and unfairly treated      | Meaning of leaving, mood, threat appraisal, coping and agency  | Voice, coping, emotion regulation          |
| Relational                 | Escalating mother-D2 conflict. D1 parentified | Sequence before and after absence, roles and alliances         | Communication, boundaries, repair          |
| Social / material          | Crowding, shift work, financial pressure      | Housing routines, income, caregiving and transport             | Practical relief and predictable care      |
| Ecological / temporal      | School-FSC-kin links fragmented               | Service history, attendance pattern, transition timing         | Coordinated mesosystem response            |

**Formulation: repeated absence may be maintained by interactions across domains. No single factor is sufficient on its own.**

# Safety, risk and strengths

Immediate safeguarding runs alongside a strengths-based assessment

| Priority           | Risk or uncertainty                       | Existing protection                      | Immediate action                            |
|--------------------|-------------------------------------------|------------------------------------------|---------------------------------------------|
| Location safety    | Where D2 stays and who is present         | Known maternal aunt relationship         | Verify safe location and contact plan       |
| Escalation         | Conflict precedes leaving                 | Family continues seeking help            | Agree pause, exit and return protocol       |
| Education          | Attendance and grades declining           | MOE school support exists                | Named school contact and attendance plan    |
| Sibling pattern    | D3 has begun accompanying D2              | Sibling bond can support change          | Assess D3 separately and avoid blaming D2   |
| Caregiver capacity | Crowding, shift work and financial strain | Stepfather employed. Kin network present | Practical relief and shared caregiving plan |

## Strengths that problem-focused practice may miss

D2 seeks connection with known kin. The aunt, D1's competence, the school link and prior service engagement are resources for change.

# Candidate theories and retained roles

PPCT organises the formulation. Complementary theories sharpen specific mechanisms

| Theory                   | What it explains well                            | Limit in this case                            | Retained role                      |
|--------------------------|--------------------------------------------------|-----------------------------------------------|------------------------------------|
| PPCT bioecology          | Cross-level interaction and developmental timing | Broad unless proximal processes are specified | Primary explanatory frame          |
| Structural family theory | Boundaries, hierarchy and parentification        | Can understate D2's subjective meaning        | Relational assessment              |
| Coercive family process  | How conflict and departure reinforce each other  | May narrow attention to behaviour sequences   | Maintenance mechanism              |
| Attachment theory        | Refuge-seeking and felt security                 | Retrospective inference can overreach         | Hypothesis about secure-base needs |
| Family Stress Model      | Material pressure through family processes       | Original evidence is context-specific         | Economic pathway hypothesis        |
| Functional casework      | Agency, choice, time and engagement              | Not an outcome model by itself                | Helping-process discipline         |

**Selection rule: retain a theory only when it changes an assessment question, intervention action or evaluation indicator.**

# Functional casework turns agency limits into a clear offer

Engagement protects choice while making time, role and responsibility explicit

- 1**    **Diagnosis tied to service**    Build a shared, provisional account linked to goals
- 2**    **Conscious use of time**    Name the beginning, middle and ending from the outset
- 3**    **Agency function**    Clarify what the FSC can offer and what it cannot compel
- 4**    **Structure as content**    Explore missed meetings without moralising
- 5**    **Relationship and choice**    Offer D2 a distinct voice and voluntary participation

# Evidence supports disciplined adaptation, not certainty

The intervention remains a monitored practice hypothesis

## Strong conceptual fit

PPCT explains cross-level and time-dependent processes. The Family Stress Model and attendance research identify plausible pathways.

## Mixed intervention evidence

The 2023 Campbell review found no consistent evidence that FFT outperformed comparison conditions. Certainty was rated very low.

## Context gap

Much evidence comes from populations and service systems outside Singapore. Local feasibility and cultural meaning require direct testing.

## PRACTICE STANDARD

**Use transparent goals, routine outcome monitoring and predefined review points. If the pattern does not change, revise the formulation and intervention rather than attributing failure to motivation.**

# Phase plan, responsibilities and indicators

Each phase links a person goal with an environmental change

| Phase                        | Person-directed objective              | Environment-directed objective                         | Core practice                              | Indicator                               |
|------------------------------|----------------------------------------|--------------------------------------------------------|--------------------------------------------|-----------------------------------------|
| 0. Engage<br>Weeks 0–2       | D2 names safety and preferred outcomes | Adults agree contact and return protocol               | Home visits, separate meetings, safety map | Known whereabouts and sessions attended |
| 1. Reframe<br>Weeks 3–4      | D2 identifies triggers and exceptions  | Family replaces blame with sequence language           | Relational reframing and conflict mapping  | Lower blame and shared formulation      |
| 2. Change<br>Weeks 5–12      | Coping, negotiation and self-advocacy  | Clear boundaries, repair and caregiving redistribution | Practice conversations and problem solving | Fewer escalations and shorter absences  |
| 3. Generalise<br>Weeks 13–16 | Education or vocation plan owned by D2 | School-FSC-kin plan survives case closure              | School conference and relapse prevention   | Attendance trend and warm handover      |

## Decision points

**Week 4 confirms the formulation. Week 8 triggers adaptation if early change is absent. Week 16 requires a safe route back.**

# Practice pathways, ethics and escalation

A coordinated plan requires clear ownership across family, school and statutory systems

| Need                | Lead pathway                                             | Case action                                                   | Boundary                                                                                           |
|---------------------|----------------------------------------------------------|---------------------------------------------------------------|----------------------------------------------------------------------------------------------------|
| Attendance          | School counsellor / Student Welfare Officer              | Named contact, attendance review, reintegration plan          | Secondary-school response is supportive. Do not misstate primary-school compulsory education rules |
| Family intervention | Family Service Centre                                    | Home-based engagement, family sessions, case coordination     | Consent, confidentiality and agency mandate remain explicit                                        |
| Financial pressure  | Social Service Office / ComCare                          | Reassess income and practical needs, then coordinate benefits | Eligibility requires current assessment                                                            |
| Kin support         | Maternal aunt and family network                         | Safe-stay and return agreement with shared caregiving         | Verify safety and D2's wishes                                                                      |
| Protection concerns | Agency safeguarding process / CPS where thresholds apply | Document, consult and escalate according to current protocol  | Absence alone does not establish statutory grounds                                                 |

**Ethical centre: D2 participates as a young person with voice and evolving autonomy. Safety duties and caregiver responsibilities remain active.**

# Appendix: How to read the supporting material

Context for the formulation and evidence appraisal

These supplementary slides should be read alongside the main deck. They extend the biopsychosocial ecological formulation through Bronfenbrenner's PPCT model and show how research evidence supports disciplined adaptation to this family's circumstances.

**The formulation remains probabilistic rather than deterministic. The valence of likely outcomes can change as proximal processes, relationships and environmental conditions change.**

*Conceptual anchor: Bronfenbrenner and Morris (2006). Read with the evidence appraisal and limitations in the main deck.*